

# Chapter 17 (Revised): The Future of Healthcare Transformation — 2026 and Beyond

**Updated April 2026: The Federal Landscape Has Changed. The Transformation Imperative Has Not.**

*Sources: KFF analyses of One Big Beautiful Bill Act / H.R. 1 Medicaid cuts (2025-2026); Chartis 2026 State of Rural Health; Peterson-KFF Health System Tracker (March 2026); Pew Charitable Trusts, New Federal Medicaid Policies (January 2026); CMS BALANCE Model; Vermont AHEAD State Agreement; Vermont RHT Program Application; Oliver Wyman Act 167 Report; AHS Transformation Reports.*

## Where the System Is in April 2026 — A Necessary Update

Previous iterations of this chapter described the American healthcare policy environment as characterized by incremental payment reform, expanding but incomplete value-based care penetration, and policy uncertainty driven by legislative gridlock. That characterization was accurate through 2024. It is no longer accurate.

On July 4, 2025, President Trump signed H.R. 1 — the One Big Beautiful Bill Act — into law. The legislation's healthcare provisions represent the largest single reduction in federal health program spending in American history: CBO estimates \$911 billion in Medicaid spending reductions over ten years, resulting in an estimated 10 million additional uninsured Americans by 2034. The law imposes work and community engagement requirements on Medicaid expansion enrollees beginning in late 2026, requires eligibility redeterminations every six months instead of annually, caps state provider taxes, restricts state-directed payments, and phases out financing mechanisms that states have used for decades to fund their share of Medicaid costs.

Simultaneously, recognizing the devastating impact these cuts would have on rural hospitals, Congress included a \$50 billion Rural Health Transformation Program in the same legislation — providing \$10 billion annually from FY2026 through FY2030 to states for rural health infrastructure investment. Vermont received \$195 million of this fund in December 2025. All 50 states received awards.

The juxtaposition defines the current policy moment: the largest healthcare coverage contraction in American history, partially offset by a historic but time-limited and front-loaded capital investment in rural health infrastructure. Understanding the interaction between these two forces — the Medicaid contraction and the RHT investment — is essential for every healthcare leader, policymaker, and analyst working in the American system today.

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|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <div>Federal Medicaid spending reduction, H.R. 1 (10 years)</div> <div>\$911B</div> <div>CBO estimate; produces 10 million more uninsured by 2034</div>                     | <div>Rural Medicaid spending reduction (10 years)</div> <div>\$137B</div> <div>KFF estimate; rural areas disproportionately affected — Medicaid covers 1 in 4 rural adults</div> |
| <div>Rural Health Transformation Program total (5 years)</div> <div>\$50B</div> <div>\$10B/year FY2026-FY2030; temporary, front-loaded; does NOT offset Medicaid cuts</div> | <div>Rural hospitals currently operating at a loss</div> <div>&gt;40%</div> <div>Before Medicaid cuts; 52% in non-Medicaid-expansion states (Chartis 2026)</div>                 |

Figure 17.1 — The 2025 federal healthcare policy inflection point. Sources: KFF; CBO; Chartis 2026 State of Rural Health.

The fiscal arithmetic is unforgiving. KFF's analysis is explicit: the RHT Program is temporary and front-loaded, while nearly two-thirds of the Medicaid spending reductions are backloaded after FY2030. The rural health fund provides \$50 billion over five years ending in 2030; the Medicaid cuts accelerate after 2030, continuing for the remaining five years of the 10-year budget window with no offsetting capital fund. Georgetown University's Center for Children and Families has documented that the RHT fund's federal guidance emphasizes investments in technology and new activities — not backfilling provider losses — and caps payments to providers at 15% of funds awarded to each state. It is an infrastructure fund, not a revenue replacement.

For Vermont, this federal landscape has direct operational consequences. Vermont's Medicaid program — which covers approximately 19% of the state's insured population and is a significant revenue source for all 14 hospitals — will face financial pressure from the Medicaid cuts beginning in 2026-2027. The work requirement implementation, beginning in late 2026, will add administrative burden on Vermont DVHA and risk some coverage losses among the expansion population. Vermont's 14 hospitals, already financially distressed, face additional revenue pressure from Medicaid reimbursement changes at the same time they are trying to execute transformation plans.

Vermont's response to this federal pressure is the correct one: AHS Secretary Samuelson's framing — the RHT funding is a tool for executing reform, not a substitute for reform — is the analytical frame every state should adopt. Vermont is using the \$195 million for infrastructure investments that reduce ongoing structural costs: AI scribes that expand effective provider capacity, RPM that prevents hospitalizations, broadband and EMS that enable the regionalization Vermont needs. If executed well, these investments position Vermont to absorb the Medicaid revenue reductions through efficiency gains rather than service cuts. If executed poorly — if the capital is used to sustain unsustainable operations rather than transform them — Vermont will face the Medicaid cuts in 2030-2031 without the structural improvements needed to survive them.

## **Five Forces Shaping the Next Decade — Updated for 2026**

### **Force 1: The Medicaid-RHT Tension — The Defining Policy Dynamic**

The One Big Beautiful Bill Act creates a structural tension that will define American rural healthcare for the next decade: significant, permanent reductions in Medicaid revenue arriving after the expiration of a temporary, one-time capital fund. Every state that received RHT funds faces the same fundamental question: can we use this capital to build a system efficient enough to survive the Medicaid revenue reductions that arrive after the fund expires?

For most rural hospital systems, the honest answer is: possibly, if the capital is invested wisely, the structural changes are executed on the available timeline, and state-level policy responses are adequate. The states most likely to succeed are those that — like Vermont — had structural reform plans in place before the RHT Program was created, are using RHT funds as capital for pre-planned transformation rather than as operating support, and have the regulatory and governance infrastructure (Vermont's GMCB and AHS) to execute transformation systematically rather than hospital-by-hospital.

The states most at risk are those that use RHT funds for revenue replacement rather than structural change, that lack the regulatory framework to manage hospital network transformation, and that face the 2030 Medicaid revenue cliff without having made the structural changes that would allow them to manage it. Chartis estimates that 417 rural hospitals are currently vulnerable to closure nationally — and the Medicaid cuts that take effect after 2027, combined with the expiration of the RHT fund after 2030, will intensify pressure on the most vulnerable facilities.

### **Force 2: The AI Clinical Transformation — Accelerating Faster Than Governance**

Artificial intelligence is moving from healthcare's technology periphery toward its clinical center faster than any governance framework has been able to track. In 2026, ambient

clinical intelligence — AI systems that listen to clinical encounters and generate structured documentation automatically — has moved from early adopter to mainstream consideration. Physicians at Hattiesburg Clinic and similar practices nationally have reported significant reductions in documentation burden from AI scribe tools. Vermont's RHT Program explicitly funds AI scribe grants for rural and independent practices.

The next clinical AI wave — predictive population health at the individual level, diagnostic AI becoming standard in high-volume imaging and pathology, AI-enabled triage reducing unnecessary ED utilization — is 18-36 months behind ambient documentation. The convergence of FHIR-based data access, large language models, and all-payer claims databases will enable risk stratification that predicts not just which patients are high-risk but which specific clinical events — hospitalization, medication non-adherence, crisis escalation — care management programs should prevent.

AI governance is the critical gap. The FDA, ONC, and CMS are developing AI governance frameworks, but deployment of clinical AI tools is outpacing governance development at every level. Vermont's RHT AI investments — without explicit AI governance requirements as conditions of grant funding — risk deploying tools that have not been tested for differential performance across Vermont's equity populations. The Northeast Kingdom's elderly, rural Medicaid patients may perform differently in AI risk stratification models trained on national commercial data. Vermont's CIN-based AI governance framework, developed in Chapter 14, is the organizational response to this challenge.

### **Force 3: The Demographic Reckoning — No Longer a Future Problem**

Vermont's Oliver Wyman analysis projected 65+ population exceeding 30% of the state's total by 2040. As of 2026, Vermont is 14 years from that milestone — but the healthcare system consequences are not 14 years away. They are arriving now, in the form of rising Medicare enrollment, deteriorating hospital payer mix, increasing long-term care demand that existing capacity cannot meet, and a growing population of dual-eligible Medicare/Medicaid patients whose complex needs are the highest cost and most poorly coordinated care challenge in the system.

Vermont is the national canary for demographic healthcare stress. But by 2035, the challenge Vermont faces today will be present in every Northeastern state, most Midwestern states, and increasingly nationally. The Baby Boom generation that is fully in Medicare's coverage by 2030 will be in its 80s by 2040 — the frailty, dementia, and multi-morbidity phase of the demographic curve that drives the highest costs and the most inadequate care models. Vermont's efforts to develop dementia care infrastructure, PACE programs, and HCBS capacity are investments in solving a problem that will be national by 2040.

### **Force 4: The Drug Pricing Reckoning — Complexity Compounding**

The GLP-1 agonist spending wave represents the most consequential near-term pharmaceutical policy challenge for state healthcare systems. At full obesity indication coverage for Medicaid and Medicare, GLP-1s would add hundreds of billions of dollars annually to program spending — simultaneously offering potentially transformative reductions in the chronic disease burden that drives the highest healthcare utilization. The BALANCE Model (December 2025) is the federal attempt to thread this needle: negotiating lower GLP-1 prices with manufacturers to enable coverage expansion without fiscal catastrophe.

Vermont's DVHA must make a BALANCE participation decision in 2026 that balances short-term budget pressure against long-term population health investment. The long-term argument for coverage is strong: Vermont's Medicaid population has high obesity rates, high diabetes prevalence, and high cardiovascular disease risk — exactly the population where GLP-1 efficacy is most demonstrated. Reducing obesity-driven chronic disease in this population would reduce the Medicaid spending on hospitalizations, specialist care, and long-term care that is far more expensive than the drugs themselves. But Vermont's near-term Medicaid budget is already under pressure from the federal cuts, making any new high-cost coverage decision politically and fiscally difficult.

The IRA drug price negotiation — semaglutide (Ozempic, Wegovy, Rybelsus) selected for 2025 negotiations with negotiated prices effective in 2027 at \$274 for a 30-day supply, down from over \$1,000 — will meaningfully reduce the fiscal cost of GLP-1 coverage expansion if states and plans participate in BALANCE. Vermont should evaluate BALANCE participation explicitly as an equity and population health policy decision, not purely as a budget decision.

**Force 5: The Value-Based Care Tipping Point — Still Approaching**

Despite 15 years of payment reform effort, the majority of U.S. healthcare spending remains in fee-for-service arrangements. VBC penetration is real — approximately 58% of commercially insured Americans are in some form of value-based arrangement — but has not yet achieved the tipping point at which FFS becomes the minority mode of payment. Vermont's mandatory RBP and global budget approach is a direct legislative response to the failure of voluntary VBC to achieve this tipping point.

The AHEAD Model — now operating in six states (Maryland, Connecticut, Hawaii, Vermont, Rhode Island, and New York) — represents the most serious federal commitment to all-payer total cost of care accountability since CMMI's founding. If Vermont's Cohort 2 participation (beginning January 2027) produces documented savings and quality improvement by 2030, the political and policy case for expanding AHEAD to additional states strengthens significantly. Vermont's performance under AHEAD is not just a Vermont policy question — it is national evidence for or against the most ambitious federal payment reform currently operating.

**The Six-Pillar Forecast: 2026-2035 Updated**

| Pillar     | 2026 starting point                                                                                                                                                                  | Primary headwinds                                                                                                                                                                                             | Primary tailwinds                                                                                                                                                                                                  | 2035 HTR assessment                                                                                                                                                                                            |
|------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Policy     | Vermont: Acts 167/68/AHEAD creating most complete state-level reform mandate in the country. National: H.R. 1 Medicaid cuts creating coverage contraction; RHT Program front-loaded. | Medicaid work requirements (late 2026) adding administrative burden and coverage risk. Federal healthcare policy instability. State budget pressure from provider tax restrictions.                           | AHEAD Model expanding (6 states). Vermont's mandatory RBP/global budget as policy template. Price transparency momentum. BALANCE Model GLP-1 access expansion.                                                     | Moderate progress nationally; Vermont-specific progress high if execution holds. Mandatory APM participation remains the critical unresolved policy question for national transformation acceleration.         |
| Economics  | Vermont: RBP being implemented (FY2027); global budgets mandated (FY2028). National: VBC at ~58% commercial penetration; global budgets outside Vermont/Maryland very limited.       | H.R. 1 Medicaid revenue reductions affecting hospital financial positions. GLP-1 spending wave threatening to overwhelm pharmaceutical savings from IRA negotiations. Commercial premium pressure continuing. | Vermont RBP producing documented savings already (\$230M in FY2026 regulatory actions). IRA GLP-1 price negotiation (effective 2027). Maryland AHEAD results validating global budget savings. ACO REACH maturing. | Vermont: strong economics progress if global budgets implemented on schedule. National: VBC reaches 70-75% commercial by 2030; global budgets beyond Vermont/Maryland remain limited without mandatory policy. |
| Technology | Vermont: VHCURES solid; VITL/HIE                                                                                                                                                     | Two-year data lag in VHCURES limits real-time                                                                                                                                                                 | Vermont CIN providing shared analytics and AI                                                                                                                                                                      | Technology: fastest-improving pillar nationally.                                                                                                                                                               |

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|            | voluntary and incomplete; analytics vendor being procured; CIN under development. National: FHIR mandate improving interoperability; AI deployment outpacing governance.                                                                              | management. FHIR compliance incomplete at smaller community hospitals. AI governance frameworks absent in most states.                                                                                                                                                         | governance infrastructure. RHT IT advance grants funding FHIR-capable upgrades at smaller facilities. Ambient AI reducing documentation burden nationally. BALANCE Model enabling GLP-1 utilization data collection.                                                                     | FHIR interoperability becoming operationally complete for major EHR systems by 2028. AI deployment rapid; governance lagging — patient safety and equity risk if governance does not catch up.                                                                          |
| Clinical   | Vermont: Blueprint strong; CoCM/MHI pilot effective but not permanently funded; CCBHC expanding; PACE gap; primary care productivity constraint. National: Behavioral health access crisis worsening nationally.                                      | MHI behavioral health integration sustainability uncertain without AHEAD EAST Fund commitment. Primary care workforce shortage intensifying (370 FTE gap by 2030 in Vermont alone). Medicaid cuts threatening safety-net primary care providers.                               | Blueprint PCMH infrastructure among strongest in the country. CCBHC expansion adding 5 new entities by July 2026. AHEAD EAST Fund designed specifically to invest in primary care and behavioral health. CoCM Medicare billing codes enabling sustainable financing.                     | Clinical: steady improvement nationally in primary care quality; behavioral health integration accelerating. Vermont-specific: Blueprint + AHEAD primary care investment strongest in country; behavioral health integration is the critical clinical quality variable. |
| Equity     | Vermont: geographic disparities documented; BIPOC disparities smaller but real; SDOH screening operational; GLP-1 Medicaid coverage gap; Northeast Kingdom access vulnerability. National: racial disparities not narrowing despite policy attention. | H.R. 1 Medicaid cuts disproportionately affecting low-income and rural populations — amplifying existing equity gaps. Federal funding reductions for health equity programs (VDH reported \$7M retracted). GLP-1 access gap acutely affecting Medicaid populations.            | Vermont BALANCE Model participation could close GLP-1 equity gap for Medicaid enrollees. CCBHC access policy (serve all regardless of insurance). Blueprint HRSN universal screening building SDOH data infrastructure. Act 68 equity requirements embedded in accountability framework. | Equity: highest-risk pillar nationally and in Vermont. Without explicit GLP-1 coverage expansion, SDOH investment, and Northeast Kingdom access protection, Vermont's equity gaps will widen as Medicaid cuts reduce coverage for the populations with greatest need.   |
| Operations | Vermont: RHRC engagement completed; hospital transformation plans in development; AHS restructuring underway; PMO being established; CIN under development. National: Administrative complexity not declining despite FHIR and prior                  | 14 hospital transformation plans at varying stages; analytics vendor not yet operational. AHS restructuring requiring staff and organizational capacity that must be built. RHT Program implementation requiring project management infrastructure Vermont is actively hiring. | \$195M RHT Program providing capital for transformation infrastructure. All 14 hospitals engaged in transformation planning. GMCB capacity added under Act 68. Joint AHS-GMCB analytics vendor procurement underway. Vermont CIN providing shared services infrastructure.               | Operations: transformation underway in Vermont — the question is whether execution pace matches statutory deadlines. National: administrative simplification progressing slowly; meaningful reduction in administrative burden requires                                 |



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|  | authorization reform. |  |  | federal prior authorization reform that has not yet passed. |
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Figure 17.2 — Six-pillar forecast 2026-2035: Vermont and national. Sources: HTR analysis; sources cited throughout this chapter.

## Vermont's 10-Year Trajectory: Two Scenarios

Vermont's healthcare transformation trajectory from 2026 to 2035 is genuinely uncertain in ways that earlier versions of this chapter could not fully acknowledge — because the federal policy environment has changed materially since 2024. The following two scenarios represent the range of plausible Vermont outcomes, not extreme predictions.

### The Transformation Success Scenario

In the success scenario, Vermont executes its transformation agenda on or near the statutory timelines, uses its capital wisely, and demonstrates by 2030 that a small rural state can achieve genuine all-payer system transformation.

- FY2027: RBP implemented; hospital commercial rates declining toward benchmark; premium growth slows for the first time in a decade. First concrete evidence that mandatory price regulation can reduce insurance premiums without hospital closures.
- FY2028: Non-CAH hospital global budgets operational; AHEAD Cohort 2 in full operation; Vermont's hospital sector begins demonstrating the population health management improvements — reduced avoidable hospitalizations, improved chronic disease management — that global budgets are designed to incentivize.
- 2028: AHS delivers Statewide Strategic Plan with specific, quantified commitments for each pillar through 2031; Vermont's hospital network has been rationalized to a sustainable tier structure; Northeast Kingdom access protected through REH conversions paired with RHT-funded EMS and telehealth.
- 2030: RHT Program capital fully deployed; Vermont hospital sector stabilized with 10-12 sustainable facilities and 2-4 REH or CACC conversions; Medicaid revenue reductions from H.R. 1 absorbed through efficiency gains from transformation; per-capita healthcare cost growth below GDP growth for first time.
- 2031-2035: Vermont serves as national template; AHEAD model expands to additional states using Vermont's documented results as evidence base; Vermont's Blueprint + AHEAD primary care investment produces measurable population health improvements in chronic disease management; AHS restructuring model studied by other states facing comparable transformation mandates.

### The Partial Transformation Scenario

In the partial transformation scenario, Vermont makes meaningful progress but faces execution gaps that prevent full realization of the transformation agenda by the statutory deadlines.

- FY2027: RBP implemented but contested — UVMMC and other large systems seek legislative relief from specific rate provisions; GMCB maintains implementation but at a modified glide path that delays full impact.
- FY2028: Global budget methodology not finalized for non-CAH hospitals due to GMCB resource constraints and AHEAD implementation complexity; deadline extended; transformation planning delayed.
- 2028: AHS delivers Statewide Strategic Plan but it is descriptive rather than prescriptive — identifying the right questions without making the specific structural commitments that accountability requires. Northeast Kingdom hospital transition planning delayed pending equity impact analysis.

- 2030: RHT Program capital deployed but partially misdirected toward operating support rather than structural transformation; Medicaid revenue reductions begin arriving; several CAH hospitals facing renewed financial distress without the structural transformation that would have enabled them to absorb the cuts.
- 2031-2035: Vermont's transformation is real but incomplete; the system is more sustainable than the status quo trajectory would have produced, but costs remain above what full transformation would achieve; a second reform wave required to address the gaps left by the first.

Neither scenario is inevitable. The success scenario requires execution discipline, political commitment, and the organizational capacity that Vermont is actively building but has not yet fully developed. The partial transformation scenario requires execution failures that are preventable with adequate investment in implementation infrastructure. The analytical framework this book provides — and the specific organizational design the AHS Restructuring chapter prescribes — is designed to make the success scenario more likely.

*"The RHT funding is a tool that lets us put affordability plans into action. This is not our health care reform plan. This is an opportunity that we'll use to support all of the health care reform work that we're already doing in the state."*

— Jenney Samuelson, Vermont AHS Secretary, January 2026

Vermont as National Template — The Decisions That Will Determine Outcomes

Vermont's transformation will produce a set of documented answers to questions that every state will eventually face. The national significance of Vermont's experiment lies not in Vermont's size but in its head start — and in the quality of the analytical record it is creating. The following decisions, made in Vermont over the next five years, will produce the evidence base that shapes the national policy toolkit for the decade after.

| Decision                                                                                  | Vermont timeline                                 | National implication if Vermont succeeds                                                                                                                         | National implication if Vermont struggles                                                                                                                                                  |
|-------------------------------------------------------------------------------------------|--------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Will mandatory all-payer RBP reduce hospital prices without reducing access?              | FY2027 implementation; first evidence FY2028     | Confirms that mandatory hospital price regulation can close the price gap without the hospital closures opponents predict; accelerates state adoption nationally | Provides ammunition for hospital system opponents of RBP; may delay national adoption by 5-10 years                                                                                        |
| Can hospital global budgets change hospital behavior toward population health management? | FY2028 global budgets; AHEAD data from 2027-2030 | Demonstrates that global budgets produce the population health improvements Maryland demonstrated but that skeptics attribute to Maryland-specific conditions    | Suggests that global budgets without adequate primary care and SDOH infrastructure do not produce population health improvement; strengthens case for community investment as prerequisite |
| Can AHS execute a comprehensive Statewide Strategic Plan by December 2028?                | December 2028 statutory deadline                 | Proves that a state agency can successfully restructure itself as a population health system operator; provides organizational                                   | Reveals organizational capacity constraints that other states will need to address before attempting comparable mandates; valuable                                                         |

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|                                                                                                  |                                                                      | design template for other states                                                                                                                                          | learning regardless of outcome                                                                                                                                                        |
| Can the RHT Program capital be used for structural transformation rather than operating support? | FY2026-FY2030 spending window                                        | Demonstrates that one-time capital investment can produce durable structural change in rural healthcare — the fundamental question about whether the \$50B was well spent | Suggests that rural healthcare structural challenges require ongoing revenue support, not just capital investment; major policy implication for post-2030 federal rural health policy |
| Will Vermont's AHEAD participation demonstrate savings and quality improvement?                  | AHEAD performance data from 2027 onward; CMS evaluation through 2035 | Builds the national evidence base for state-level total cost of care accountability; may accelerate AHEAD expansion to additional cohorts                                 | May lead CMS to modify or restructure AHEAD if Vermont's experience reveals design flaws; still valuable as evidence                                                                  |

Figure 17.3 — Vermont's transformation decisions and their national policy implications. Sources: HTR analysis; Vermont AHEAD State Agreement; Act 68 of 2025.

A Closing Note: What This Book Is Betting On

Every analytical framework contains implicit bets about what matters and what does not. It is worth making this book's bets explicit.

This book bets that structural change is possible — that the combination of strong enough financial pressure, adequate capital investment, political will sufficient to maintain mandatory policy over hospital system resistance, and organizational capacity built over years of preparation can produce a genuinely different Vermont healthcare system by 2035. This is not a certainty. It is the most plausible outcome given Vermont's specific combination of factors: a small, transparent, high-capacity state government; a hospital sector that is financially distressed enough to be willing to transform rather than resist; a regulatory body (GMCB) with genuine authority and demonstrated willingness to use it; a reform-minded legislature that has sustained a coherent policy agenda through multiple administrations; and now capital — \$195 million in federal investment aligned to the pre-existing reform plan.

This book also bets that the transformations described here are transferable — that the analytical framework, the payment mechanisms, the clinical models, and the organizational designs are not Vermont-specific solutions to Vermont-specific problems, but general-purpose tools for the structural healthcare challenges that every American state will eventually face. Vermont's specificity — its small size, its transparency, its policy history — makes it an unusually good teaching case. But the lesson is generalizable.

The deepest bet this book makes is about the relationship between analysis and action. Healthcare transformation fails not primarily because of analytical errors — the evidence base for what needs to change is strong and consistent — but because analysis does not automatically produce the institutional capacity, political will, and organizational execution required to implement what the evidence recommends. This book provides the analytical framework. The institutional capacity, political will, and organizational execution are built by the people who read it and do the work.

Vermont's transformation is happening because specific decision-makers — legislators, regulators, hospital executives, clinicians, community health workers, and patients — are making specific decisions at specific moments. The analytical framework this book provides is in service of better decisions. What those decisions produce — a transformed system or a partial transformation, a national template or a cautionary tale — belongs to the people who make them.



Sources: KFF, *A Closer Look at the \$50 Billion Rural Health Fund in the New Reconciliation Law* (September 2025); KFF, *How Might Federal Medicaid Cuts in the Enacted Reconciliation Package Affect Rural Areas?*; KFF, *What to Know About the BALANCE Model for GLP-1s in Medicare and Medicaid* (2026); Chartis, *2026 State of Rural Health* (February 2026); Peterson-KFF Health System Tracker, *Eight Trends Shaping 2026 Healthcare Costs* (March 2026); Pew Charitable Trusts, *New Federal Medicaid Policies Compound State Budget Pressures* (January 2026); Georgetown Center for Children and Families, *States Beginning to Grapple with Federal Medicaid Cuts Impact on Rural Health Care* (March 2026); CMS BALANCE Model announcement (December 23, 2025); Vermont AHS Secretary Samuelson, remarks January 29, 2026; Oliver Wyman Act 167 Community Engagement: Recommendations (2024); Vermont AHEAD State Agreement (January 2025); Vermont RHT Program Application (November 2025).